

ADDRESS THE DYING PATIENT'S NEEDS

The patient with a life-limiting illness is dying if s/he is deteriorating and ≥ 2 of: bed bound, decreased consciousness, only able to sip fluid or unable to take tablets. A doctor should confirm this.

Assess the dying patient's needs every 4 hours

Assess	Note
Symptoms	Assess for noisy/difficulty breathing, agitation, pain, constipation, diarrhoea, nausea/vomiting and abdominal cramps. If present, manage below.
Current care	- Assess current medication and discontinue non-essential medications. - Assess patient's ongoing need for tests in discussion with patient/carer and health care team. - Consider switching medication route if unable to swallow orally to subcutaneous.
Intake	Check with carer/family what patient's fluids/food intake needs are and whether fluids/food is needed or necessary.
Psychological well-being	Ask how patient and carer are coping and what support and/or spiritual care is needed. If carer unable to cope at home, refer patient to hospital/hospice.
Mouth	Check oral hygiene. Ensure patient's mouth is moist and clean. Consider using glycerine to keep lips/mouth moist.
Personal hygiene	Check skin care, clean eyes and change of clothing according to patient's needs.

Advise the dying patient and carer

- Ensure patient and/or carer is aware that patient is dying and that carer/family have been referred to social worker and community health worker.
- Educate carer/family that food/fluids are for comfort only, will not prolong life and a reduced need for food/fluids is part of the normal dying process.
- Advise that investigations and curative treatments like antibiotics may no longer be indicated and will be kept to a minimum according to patient's care plan.
- Discuss with patient and carer: preferred place of death (home, hospice or hospital), how family are to be informed of impending death, what to do in the event of death.
- Discuss patient's wishes, feelings, faith, beliefs and values. Discuss patient's needs now, at death and after death. Listen and respond to patient/carer's worries/fears.
- Ensure patient and/or carer/family receive full explanation and express understanding of current plan of care. Identify and document any concerns about current plan of care.

Treat the dying patient

- If noisy breathing, excessive secretions likely: try changing position.
- If difficulty breathing, use fan or open window/s. Give **morphine hydrochloride** solution 2.5-5mg. Increase slowly as needed.
- If urinary retention, insert urethral catheter.
- If pain, constipation, diarrhoea, nausea/vomiting or abdominal cramps, manage ➔ 171.
- If agitated:
 - First assess for and manage pain, urinary retention, constipation and dehydration.
 - If none of above, consider changing position and give **diazepam** 5mg (or 2.5mg if liver failure). If no better, repeat dose.

Review the dying patient

- Doctor to review every 3 days or sooner if carer/family concerned about current plan of care or if patient's conscious level, functional ability, oral intake or mobility improves.
- If carer/family unable to cope at home or difficulty breathing relieved by oxygen, refer to hospital/hospice if available.
- If unsure, discuss with palliative care specialist.

Diagnose death if:

No carotid pulse in neck for 2 minutes *and* no heart sounds for 2 minutes *and* no breath sounds or chest movement for 2 minutes *and* pupils are fixed, dilated and do not respond to light.